MEDCONTRACTINTEL

VOL. II · NEGOTIATION

Volume II of the MedContractIntel Compensation Series

The Negotiation Script Pack

Six Word-for-Word Scripts, Six Email Templates, and an Objection-Handling Guide for Internal Medicine Physicians

Data. Leverage. Fair Pay.

MedContractIntel Compensation Series · Volume II
Internal Medicine · Outpatient + Hospitalist

Internal Medicine Edition · 2026

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How to use this book

Negotiation Script Pack is structured to be read in order or used as a reference. Each chapter stands on its own, but the chapters build on each other. Here's where to look for what you need most.

Read the Foreword first.

It explains why most physicians negotiate poorly and the three gaps — language, data, prep — this book is built to close. Five minutes. Sets the frame for everything else.

Pick the script that matches your situation.

Script 1 is for outpatient renewal. Scripts 2 and 4 are deflection-counters. Script 3 is for panel growth. Script 5 is for nocturnists. Script 6 is the longest-leverage clause-add. Read the one you need; come back for the rest later.

Memorize the relevant objection responses.

Part II gives eight word-for-word counters to the most common employer deflections. Say them as written. Soft language costs you money.

Print the Pre-Negotiation Worksheet (back of book).

Fill it in 24 hours before the meeting. If you cannot complete every field, you are not ready. The act of filling it in is the preparation.

Pair this with the wRVU Playbook (Volume I).

The Playbook teaches the math behind the asks in this book. If a script's data references feel unfamiliar, the corresponding chapter of the Playbook will resolve them.

Scan icons

The body uses four small marks to help you skim. Each labels what kind of paragraph it leads.

■	**DATA**	A key benchmark or specific dollar figure you can cite
▲	**WATCH**	A common employer trap or below-market structural pattern
★	**WIN**	A move that compounds in your favor over the contract life
■	**DO**	A concrete action — pull this number, request this clause, do this thing

Cross-references to other titles in the MedContractIntel Compensation Series (the Internal Medicine Physician Contract Red Flag Checklist, the Negotiation Script Pack, the wRVU Playbook, and the Hospitalist Shift Economics Analyzer) appear in the body where relevant. Each title is independent but designed to complement the others.

Foreword

Why Most IM Physicians Negotiate Poorly

Most Internal Medicine physicians walk into contract conversations missing three things: the language, the data, and the objection prep. You have been trained to treat people, not to negotiate against a compensation analyst who runs this conversation fifty times a year. The asymmetry is not your fault — it is structural. This book is designed to close it.

▲ The first gap is **language.** Physicians tend to describe compensation in feeling-terms: "I think I'm underpaid," "It seems low for what I do," "The work has gotten harder." Compensation analysts do not respond to feelings. They respond to benchmarks. *"I feel underpaid"* gets a polite nod and no movement. *"Published 2025 surveys place the median for outpatient Internal Medicine around $308,000. My current total compensation is $272,000. I am below median for my specialty and setting"* is a different conversation. One is a complaint. The other is a request grounded in data the employer's own analysts use to set pay. The second one moves numbers.

■ The second gap is **data.** You need four specific numbers ready before you open your mouth: your current total compensation (base plus productivity plus quality, fully loaded), your current wRVU production (last twelve months), the published 2025 median and 75th-percentile total compensation for your specialty, and your per-wRVU compensation ratio. If you do not know these four numbers, you are not ready to negotiate. You are ready to be told no.

■ The third gap is **objection prep.** The compensation analyst will have six to eight standard deflections ready. *"The market is soft." "We pay everyone the same." "That data doesn't apply to us." "Quality bonuses cover the gap." "You have great benefits."* These are not arguments — they are scripts, rehearsed and deployed to end the conversation. If you have not prepared responses in advance, each one closes a door. Part II of this book gives you the counter-script for each one.

$52,000

Five-Year Cost of a $2/wRVU Shortfall

At 5,200 annual wRVUs, compounded across the contract term

There is a specific disadvantage Internal Medicine physicians face relative to other specialties. IM contracts are longer-term and more structurally complex than, for example, Emergency Medicine contracts. An EM physician typically negotiates a rate and a shift count. An IM physician negotiates a base, a production multiplier, a panel size (or a census expectation), a quality bonus structure with multiple components, call obligations, and often a capitation or attribution layer. Every one of those terms is a place where a bad number compounds over the life of the contract.

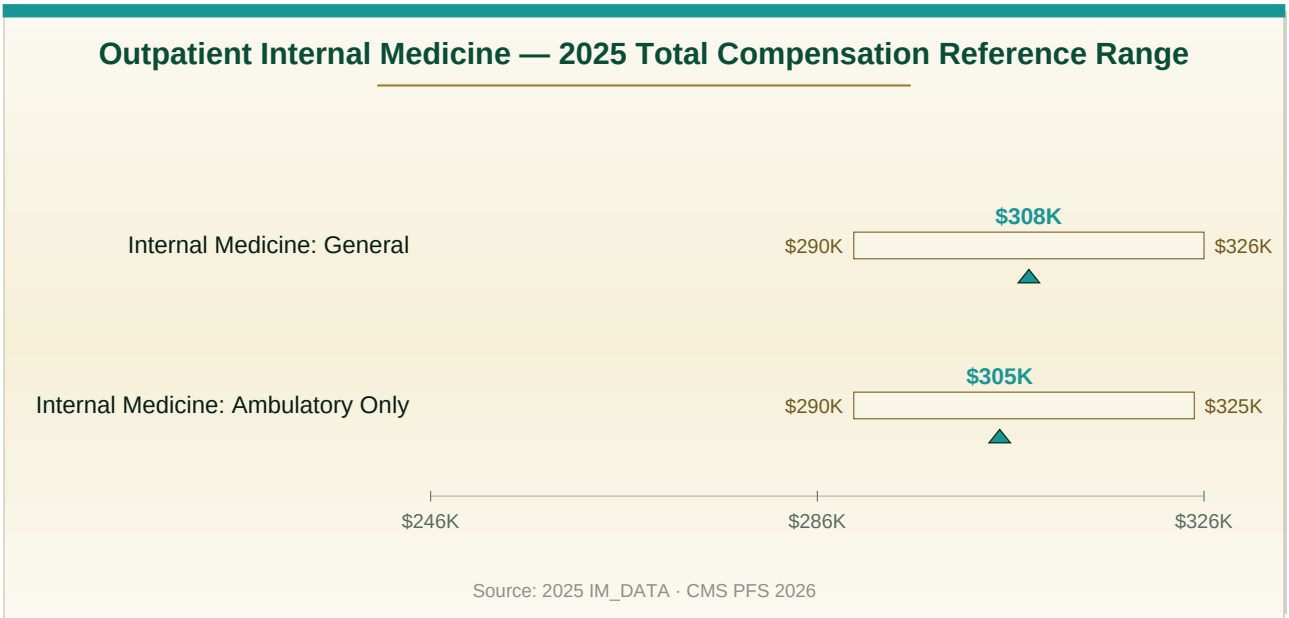
“You are not a supplicant making an ask. You are a market participant reconciling your compensation against the same data the employer uses.”

One principle sits above all the scripts in this book: **you are not asking for a favor. You are re-anchoring to market.** The employer's compensation committee sets pay by referencing the same published surveys you will cite. You are entitled to reference the same benchmarks. When you walk into the conversation, you are not a supplicant making an ask. You are a market participant reconciling your compensation against the same data the employer uses. Carry that into every conversation. It changes your posture, it changes your tone, and it changes what the other side expects you to accept.

Script 1

wRVU Multiplier Increase at Renewal (Outpatient IM)

The scenario. You are at renewal. Your current wRVU multiplier is $44. You have done your homework: published 2025 compensation surveys place the median total compensation for Internal Medicine: General around $308,000.



Your annual wRVU production is 5,100 — solidly in the 50th–60th percentile range for your specialty. At $44/wRVU, your production-tied compensation is $224,400; with base and bonuses you are at $265,000. You are below median. The conversation happens in your administrator's office, scheduled in advance, 45 minutes on the calendar.

Opening

- Do not start with pleasantries that dilute the ask. Start on topic.

> *Thank you for making time. I want to use this meeting to discuss my compensation at renewal. I've benchmarked my current contract against published 2025 compensation data and I want to walk through what I'm seeing, and what I'm asking for.*

That opening does three things: it frames the meeting as a negotiation (not a check-in), it establishes that you've done the analytical work, and it moves straight to the ask. No apologizing. No softening.

Presenting the data

Have a one-page summary in front of you. Not a long PowerPoint — one page with your current numbers, the published reference range, and the gap.

> *My current total compensation is $265,000. That puts me below the published median range for Internal Medicine: General, which sits around $290,000 to $326,000. My production last year was 5,100 wRVUs, which is at roughly the 55th percentile. My compensation percentile is below my production percentile. The gap traces to the wRVU multiplier. At $44, I'm producing at the 55th percentile but being compensated closer to the 35th. I'd like to discuss closing that gap.*

Making the specific ask

Vague asks get vague responses. Ask for a specific number.

> *I'm requesting the multiplier move from $44 to $52 at renewal. That's the rate that brings my total compensation in line with my production percentile. I'm not asking for an increase above my production — I'm asking for the multiplier to match what I'm already generating.*

If they push back

They will. The common response is some version of "we need to look at the budget" or "our multipliers are standardized." Do not retreat. Acknowledge and redirect.

> *I understand there are budget and structural considerations. I'd like to leave today with either a specific counter-offer or a timeline for when you can bring one back. Is it reasonable to expect a response within two weeks?*

Closing

You leave the meeting with either a commitment to a specific response or a counter-offer already in hand. You do not leave with "let me think about it" and no timeline.

Follow-up email template

Subject: Follow-up on contract renewal discussion

Dr. [Administrator's Last Name],

Thank you for meeting with me yesterday to discuss my contract at renewal. As I mentioned, my request is to adjust the wRVU multiplier from $44 to $52, which aligns my compensation with the production level I am currently delivering based on published 2025 compensation benchmarks.

For your reference, my annual wRVU production for the most recent twelve months was 5,100 wRVUs. This places me at approximately the 55th percentile for Internal Medicine: General in current published data. The multiplier adjustment I am requesting would bring my total compensation in line with that production percentile.

I would appreciate a response by [specific date — two weeks out]. Please let me know if you need additional production data or benchmarking detail to support the review.

Best, [Name]

Script 2

Countering "This Contract Is Non-Negotiable"

This is the most common deflection in physician contract negotiations. It is also the least accurate. A contract presented as non-negotiable almost never is — the phrase means "we have not seen pressure to move yet." Your job is to supply the pressure. Not aggressively. Precisely.

The psychological reframe

When you hear "this contract is non-negotiable," do not hear a door closing. Hear the first move in a standard opening. Your response is not to argue the point. It is to redirect.

> *I understand the standard contract reflects a framework the organization uses across physicians. I'm not asking for special terms — I'm asking to discuss whether my specific profile, production, and benchmark position warrant an adjustment within that framework. Can we walk through the specific terms where there might be flexibility?*

That response does not challenge their premise. It accepts the premise and redirects to the specific terms where movement is actually possible. Most "non-negotiable" contracts have 3–5 terms that are, in fact, negotiated every day: base compensation, sign-on bonus, CME allowance, relocation allowance, tail coverage, and the wRVU multiplier for physicians above a production threshold.

The data pivot

After the reframe, bring the conversation back to data.

> *Published 2025 compensation surveys place the median for Internal Medicine: General around $308,000. My current offer is at $272,000. I'm not asking you to abandon the standard contract — I'm asking whether there is flexibility in base, in sign-on, or in the multiplier to bring the total compensation closer to the published median. If the answer is no across all three, I understand — I'd just like to hear it specifically, not generally.*

This forces the conversation off the vague "non-negotiable" posture and onto specific, negotiable components. A refusal on one specific item is easier to accept and easier to counter than a blanket refusal.

If they hold firm

Sometimes the employer genuinely will not move. That is a real outcome, and you should be ready for it.

> *Understood. I appreciate you being direct about it. To make a good decision, I'd like two weeks to review the full package against my other options. Can we schedule a follow-up on [specific date]?*

The request for time is itself a signal. It tells the employer that you are considering alternatives, without making any explicit threat.

The follow-on ask

Whether or not the primary ask moves, you should have a secondary ask ready. Sign-on bonus, CME allowance, relocation allowance, tail coverage, termination notice period — any one of these is a smaller number for the employer to approve and a meaningful number for you.

> *If the base multiplier is firm, I'd like to ask whether the sign-on bonus or CME allowance has any flexibility. A $10,000 sign-on increase or a $3,000 CME increase would be a meaningful adjustment within the framework you've described.*

Follow-up email template

Subject: Follow-up on contract terms

[Administrator's Last Name],

Thank you for the conversation today. I understand the standard contract framework, and I am not asking for special terms — I am asking whether there is flexibility within the standard framework on specific components.

Based on our conversation, I am proposing the following adjustments for your review:

1. wRVU multiplier: increase from $[X] to $[Y], supported by published 2025 compensation data showing the median for [specialty] around $[Z].
2. Sign-on bonus: increase from $[X] to $[Y], reflecting the current market for [specialty] in this region.
3. Tail coverage: employer-paid on termination without cause.

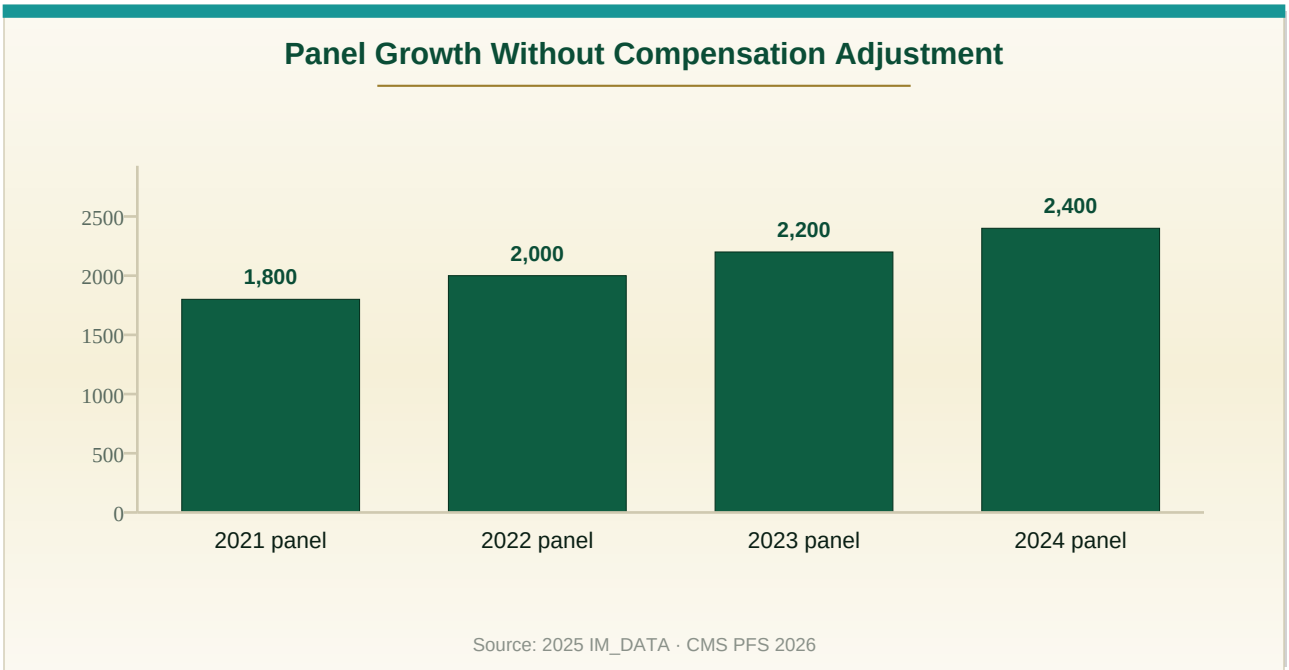
I am flexible on the combination, and I understand not every item may be available. I would appreciate a response by [date] so that I can make a decision on the full package.

Best, [Name]

Script 3

Panel Size Cap and Productivity Adjustment

The scenario. You are an outpatient IM physician with a panel of 2,400 patients. Your contract specifies no panel cap and no additional compensation for panel growth. The panel has grown from 1,800 over the past three years as physicians left the group and were not replaced. You are doing 33% more administrative work — messages, refills, prior authorizations, forms, results review — at the same pay.



Opening the conversation

Do not start by complaining about workload. Start with the data.

> *I want to discuss my panel size and the structure of my compensation. My panel has grown from 1,800 to 2,400 over the last three years. My total compensation has not changed proportionally. I'm asking to discuss either a panel cap or a per-patient-above-threshold payment structure.*

The data anchor

Widely cited primary care benchmarks place a sustainable full-time IM panel at 1,800–2,200. Panels above 2,200 are consistently associated with higher administrative burden and are frequently flagged in burnout research.

> *The widely referenced sustainable panel range for outpatient Internal Medicine is 1,800 to 2,200. My panel is at 2,400 and growing. I am not asking to shed patients — I am asking for the contract to reflect the workload.*

The specific ask

Offer two options. Employers prefer choices to ultimatums.

> *I see two ways to resolve this. Option one: cap my panel at 2,200, with any further growth distributed to other physicians or to new hires. Option two: maintain the current panel or higher, but add a per-patient-above-2,000 payment — for example, $25 per patient per year above 2,000, which would add $10,000 at a 2,400 panel and scales appropriately if the panel continues to grow. I'm open to either structure. I am not open to maintaining the status quo.*

That framing does two things. It provides the employer with a choice that feels like control. And it forecloses "no change" as an option — you have already identified that as unacceptable.

If they offer a one-time bonus instead

A common counter. Do not accept it.

> *A one-time bonus addresses the retrospective workload but does not address the ongoing structural issue. If the panel continues to grow next year, I'll be in the same conversation. I'm asking for a structural adjustment — a cap or a per-patient rate — not a one-time payment.*

If they cite administrative limits

Sometimes the employer cannot cap a panel because patient attribution is driven by a contracted payer or an ACO. That is a real constraint. The response is to accept the constraint and move to the compensation side.

> *Understood that the attribution is driven by the payer. In that case, let's focus on the per-patient-above-threshold payment, since that doesn't touch the attribution structure.*

Follow-up email template

Subject: Panel size and compensation structure

[Administrator's Last Name],

Thank you for the discussion about my panel and compensation structure. My panel has grown from 1,800 to 2,400 over the last three years without a corresponding compensation adjustment. I am requesting one of two changes, and I am flexible on which one is implemented:

1. A panel cap at 2,200, with new patients directed to other physicians or new hires as the panel is rebalanced.
2. A per-patient-above-threshold payment — for example, $25 per attributed patient per year above 2,000 — added to the compensation schedule.

Either option addresses the structural gap between my panel and my compensation. A one-time bonus is not sufficient because the issue is ongoing. I would appreciate a response by [date].

Best, [Name]

Script 4

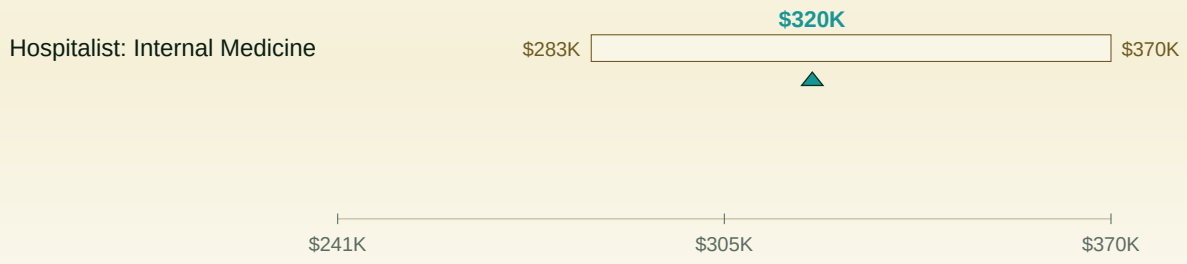
"The Hospital Budget Doesn't Allow For More"

This is the hospital-employed hospitalist version of the "non-negotiable" deflection. The response framework is the same: accept the premise, redirect to specifics, anchor to market data.

The reframe

Budget constraints are real. They are also selectively invoked. A hospital with a stated budget constraint still makes hiring decisions, still pays market rates for specialties it cannot recruit below market, and still uses market data to set pay. Your job is to bring the market data into the conversation.

Hospitalist: Internal Medicine — 2025 Reference Range



Source: 2025 IM_DATA · CMS PFS 2026

> *I understand the budget is a real constraint. Published 2025 compensation surveys place the median total compensation for Hospitalist: Internal Medicine around $320,000. My current compensation is at $[X], which is below that median. The question I'd like to discuss is not whether the hospital can exceed its budget — it's whether the hospital's budget for this role is set at a market-competitive level. If it is below the 25th percentile of published data, the budget is the structural issue, not the request.*

This response does something important: it reframes the budget question. The hospital sets its own budget. If the budget produces below-market compensation for a role the hospital needs to fill, the budget is wrong, not the request.

The data pivot

> *Published 2025 hospitalist data places the 25th percentile around $283,000, the median around $320,000, and the 75th percentile at $370,000 or above. Where is my current compensation on that distribution? If the answer is below the 25th percentile, we have a retention issue. If it's near the median, I understand the position. If it's between, there's a conversation to have about closing the gap.*

If they cite rural or regional adjustment

Sometimes the hospital will argue that national data does not apply to the local market. That may be true.

> *If local market data differs from national, let's use local data. The published surveys provide regional breakouts. I'm open to the conversation being anchored to regional data — what I'm not open to is a conversation with no benchmark at all. Can you share the specific regional benchmark the hospital uses?*

The retention frame

If all else fails, raise retention.

> *The cost of replacing a hospitalist — recruitment, locum coverage during the gap, credentialing, ramp-up — is typically $200,000 to $300,000. A $15,000 per year compensation adjustment over the next three years is less than the cost of replacement. I'd like the conversation to include that framing.*

Follow-up email template

Subject: Follow-up on hospitalist compensation

[Administrator's Last Name],

Thank you for the discussion. I want to clarify the request in writing so we're working from the same numbers.

My current total compensation is $[X]. Based on published 2025 compensation surveys, the median total compensation for Hospitalist: Internal Medicine sits around $320,000. The 25th percentile is approximately $283,000.

If the hospital's budget for this role is set below the 25th percentile of national data, I would like to understand what regional or local benchmark the budget is anchored to. If the budget is set between the 25th and 50th percentile, I am requesting movement toward the median.

I would appreciate a response by [date]. Happy to meet again if it is helpful.

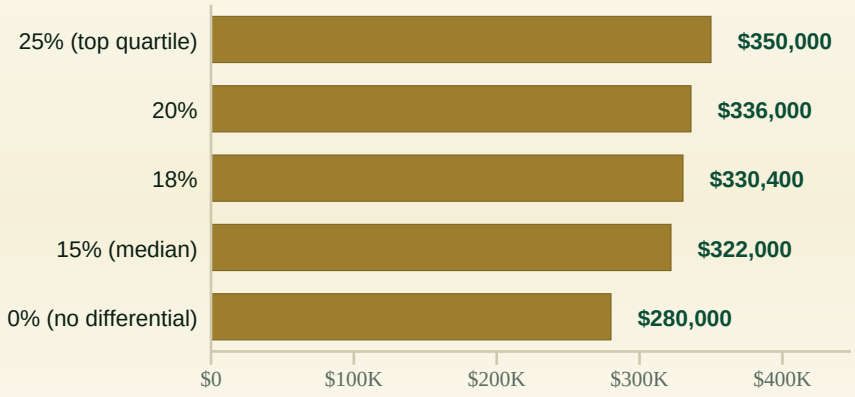
Best, [Name]

Script 5

Nocturnist Differential (Hospitalist)

The scenario. You are being offered (or currently work) a nocturnist schedule. The offered differential is either zero, or below 15%. Published 2025 hospitalist compensation surveys place the typical nocturnist differential at 15–25% above base hospitalist compensation, with median structures clustering at 15–20% and top-quartile groups paying 20–25%.

Total Compensation by Nocturnist Differential (Base $280,000)



Source: 2025 IM_DATA · CMS PFS 2026

The opening

Start with the data.

> *I want to discuss the nocturnist differential in this offer. Published 2025 surveys show the typical nocturnist compensation differential is 15 to 25 percent above base, with the median around 15 percent and top-quartile practices paying 20 to 25 percent. The differential in this offer is [X]%. I'd like to discuss closing that gap.*

The specific ask

Ask for a range, not a point.

> *My request is to move the differential to 18 to 20 percent on base. That puts the compensation above the published median and below the top-quartile range, which I think reflects both the role and the facility. On a base of $280,000, a 20% differential is $56,000 — bringing the total to $336,000.*

If they offer a one-time bonus instead

▲ This is a common counter. The employer will propose a $10,000 sign-on or retention bonus to "bridge the gap." Do not accept.

> *A one-time bonus addresses year one. The schedule is ongoing. A 20% differential on base compounds across the full term of the contract. At a $280,000 base over five years, the difference between a one-time $10,000 bonus and an ongoing 20% differential is $270,000. I'm asking for the differential, not the bonus.*

That response shows you have done the math. It moves the conversation from "can we find money to make you feel valued" to "let's structure the compensation correctly."

The schedule differential counter

Some groups pay below-market compensation differentials but offer schedule differentials — fewer shifts than daytime hospitalists. If that is on the table, you may accept it. But do the math first.

> *If the differential is schedule-based rather than compensation-based, let's quantify it. How many shifts does a daytime hospitalist in this group work per year, and how many would a nocturnist work? If a nocturnist works 15% fewer shifts at the same pay, that's equivalent to a 17.6% per-shift differential, which is at the published median. If the schedule differential is less than 15%, the total package is below market.*

The "adjustment" trap

▲ Be alert to a common employer move: framing the differential as compensating for "difficulty of nights" rather than as compensation for the shifted schedule. The language matters. A differential framed as "for night work" is a rate structure. A differential framed as "for the difficulty" is a discretionary bonus that can be removed.

> *I'd like the differential written into the contract as a fixed percentage of base, applied to all nocturnist shifts. Not as a discretionary premium.*

Follow-up email template

Subject: Nocturnist differential — follow-up

[Administrator's Last Name],

Thank you for the discussion about the nocturnist role. To summarize my request in writing:

I am asking for the compensation differential to be set at 18 to 20 percent above base hospitalist compensation, written into the contract as a fixed percentage applied to all nocturnist shifts. This request is anchored to published 2025 hospitalist compensation data showing the typical nocturnist differential at 15 to 25 percent (median 15 percent, top-quartile 20–25 percent).

A one-time sign-on or retention bonus does not address the structural differential over the term of the contract. If the group's approach is a schedule differential (fewer shifts) rather than a compensation differential, I would like to quantify that against the published benchmarks to determine whether the total package is at or above the median.

I would appreciate a response by [date].

Best, [Name]

Script 6

Annual Compensation Review Mechanism

The scenario. You want to add language to the contract requiring an annual compensation review tied to published benchmark data, rather than discretionary employer-initiated adjustments. This prevents the slow erosion that happens when the market moves and your contract does not.

Why this matters

★ Compensation review clauses in most physician contracts are either absent or discretionary. *"Compensation may be reviewed annually at the employer's discretion"* is functionally no clause at all — it grants the employer permission to do what the employer could already do. What you want is a clause that obligates the review, specifies the benchmark category, and creates a mechanism for adjustment.

The opening

> *I want to discuss adding a compensation review mechanism to the contract. Not discretionary language, but a specific annual review tied to published benchmarks. This protects both of us — it ensures the compensation stays aligned with market as the market moves, and it removes the friction of having to reopen the conversation from scratch every year.*

The specific contract language to request

> *Beginning on the first anniversary of the effective date and annually thereafter, Physician's total cash compensation shall be reviewed against then-current published compensation surveys for the applicable specialty. If Physician's total cash compensation is below the median of the applicable published surveys, Physician and Employer shall meet in good faith within 30 days to discuss an adjustment. Employer retains discretion over the amount of any adjustment, but shall not unreasonably withhold consideration.*

That language does not guarantee a raise. It guarantees a review anchored to specific data and a meeting to discuss it. That is the achievable ask. An automatic-raise clause is almost never granted and not worth burning credibility to pursue.

If they push back on "good faith" language

> *Good faith is standard contract language. If the preferred phrasing is different — "meet in a commercially reasonable manner" or "review the then-current benchmark data and respond in writing" — I'm flexible on the phrasing. What I'm asking for is the review to be required, not optional, and for the benchmark to be specified.*

If they offer a discretionary clause instead

> *A discretionary clause is what the current contract effectively has. What I'm asking for is a specific trigger — a benchmark gap — that requires a conversation. Without that trigger, the review is the same as the current situation, which is that I have to initiate it myself and there is no structural requirement for the employer to engage.*

The strategic frame

★ This is one of the easier asks in a negotiation. It does not commit the employer to a specific dollar amount. It commits them to a process. Employers often grant process concessions more easily than dollar concessions, and a process concession that requires review against published benchmarks is itself a significant protection.

Follow-up email template

Subject: Proposed annual compensation review language

[Administrator's Last Name],

Thank you for being open to discussing a compensation review mechanism. The specific language I would like to propose for inclusion in the contract is:

> *Beginning on the first anniversary of the Effective Date and annually thereafter, Physician's total cash compensation shall be reviewed against then-current published compensation surveys for the applicable specialty. If Physician's total cash compensation is below the median of the applicable published surveys, Physician and Employer shall meet within 30 days to discuss an adjustment. The Employer retains discretion over the amount of any adjustment but agrees to review the benchmark data in good faith.*

This language does not commit the organization to any specific dollar adjustment. It commits to a process anchored to the same benchmark data the organization uses to set pay internally. I believe this protects both parties.

Happy to work with counsel on alternate phrasing. My priority is that the review be required and benchmark-anchored, not that the specific wording be preserved.

Best, [Name]

Part II — Defense

Objection-Handling Quick Reference

Eight of the most common employer objections in IM and hospitalist contract negotiations, and the specific response to each. These are designed to be memorized. Say them the way they are written. Do not soften them.

Objection 1: "The market is soft."

> *Published 2025 compensation surveys show Hospitalist: Internal Medicine median compensation around $320,000 — up from prior years. Recent productivity surveys show hospitalist compensation grew approximately 2.5% year-over-year while productivity grew 5.8%, which means physicians produced more per dollar of compensation, not less. The data doesn't support a soft market. If the hospital's internal data shows something different, I'd like to see it.*

Hospitalist YoY Compensation vs. Productivity

Source: 2025 IM_DATA · CMS PFS 2026

Objection 2: "We pay everyone the same."

> *I'm not asking for more than the standard for my specialty, production percentile, and experience level. I'm asking for the standard to be benchmarked against published data, and for my compensation to reflect my actual percentile position within that benchmark. If every physician in my role is below median, that is a structural issue with the standard — not a request for an exception.*

Objection 3: "You just started here."

> *Tenure is one factor in compensation, but it's not the primary factor in the published benchmarks. Those reports stratify by specialty and production, not by tenure within a specific organization. My request is anchored to the specialty and production benchmarks. If the organization uses a separate tenure-based adjustment layered on top, I'd like to understand that structure.*

Objection 4: "That data doesn't apply to us."

> *Published physician compensation surveys are the industry-standard benchmarks, used by compensation committees and fair-market-value opinions at virtually every hospital and health system. If the organization uses a different benchmark, I'd like to see it and understand why it differs from the standard. If no external benchmark is being used, that's also important to know.*

Objection 5: "We can't change the standard contract."

> *I understand there's a standard framework. I'm not asking to replace the framework — I'm asking to discuss whether the specific components within the framework have flexibility. The wRVU multiplier, sign-on bonus, CME allowance, and tail coverage are all components that get adjusted within standard contracts regularly. Which of those has flexibility, and which does not?*

Objection 6: "Quality bonuses cover the gap."

> *Let's run the math. The quality bonus structure you've described has a maximum annual payout of $[X]. If I achieve 100% of quality targets every year — which is rare — my total compensation is $[base + max bonus]. If that total is still below the published median, the quality bonus doesn't close the gap; it only narrows it. The appropriate question is whether the base plus typical achievable quality bonus puts me at median, not whether the theoretical maximum does.*

Objection 7: "You have great benefits."

> *Benefits matter, and I appreciate the package. The published total compensation benchmarks, however, already include the value of benefits — retirement contributions, CME allowances, health coverage, and so on. When I'm comparing my compensation to the published median, I'm comparing total compensation including benefits. The gap I've identified is a gap in total compensation, not base alone.*

Objection 8: "This is just how hospital employment works."

> *Hospital employment is the majority of the market, and the published surveys reflect the majority of the market. So when I anchor to those benchmarks, I'm anchoring to data that already reflects hospital employment structures. If the hospital pays below the published median for hospital-employed physicians in my specialty, that's a specific gap, not a general feature of hospital employment.*

Part III — Preparation

Pre-Negotiation Checklist

Twelve items to have ready before you enter the conversation. If any of these are missing, you are not ready.

1. **Current contract terms, summarized on one page.** Base salary, production multiplier, bonus structure, call obligations, panel size (if outpatient), shift structure (if hospitalist), termination notice, non-compete scope, tail coverage, sign-on and retention bonuses, CME allowance, PTO.
2. **Current total compensation, fully loaded.** Base plus production plus bonuses plus the employer-reported value of benefits. Most employers provide a total-compensation statement annually — if yours does not, construct one from your W-2, 1099, and benefits summary.
3. **Current wRVU production, last twelve months.** If your employer uses Epic, Cerner, or a similar EHR with a productivity dashboard, pull it. If not, request it in writing from your administrator. You need the actual number, not an estimate.
4. **Position on the published 2025 reference range.** Where does your current compensation fall relative to the published medians and percentiles for your specialty (Hospitalist: Internal Medicine, Hospitalist: Nocturnist, Internal Medicine: General, or Internal Medicine: Ambulatory Only)? Identify whether you are below, at, or above median.
5. **Panel size vs. group median or panel norm (outpatient IM).** What is your panel relative to the typical panel in your group, and relative to published sustainable panel norms (1,800–2,200 for full-time outpatient IM)?
6. **Quality bonus structure and actual performance.** What are the quality metrics that trigger your bonus? What is your performance against each in the most recent measurement period? What is the maximum possible payout and your typical actual payout?
7. **Tail coverage terms.** Is your malpractice policy occurrence or claims-made? If claims-made, who pays the tail on termination? What does the tail typically cost for your specialty in your state?
8. **Non-compete geography and duration.** What is the scope (miles) and duration (months/years) of the non-compete in your current contract? What is the enforceability in your state?
9. **Comparable offers, if any.** Have you had a conversation with a recruiter, reviewed a locum rate sheet, or received an informal offer from another group? Real alternative offers are the strongest negotiating leverage.
10. **Hospitalist-specific structural data.** For hospitalists, the relevant structural benchmarks are group size, shift structure, call mix, nocturnist differential, admitter-rounder models, and non-production incentives.

11 **YoY productivity-vs-compensation framing.** Recent industry data shows hospitalist compensation grew approximately 2.5% YoY while productivity grew 5.8%. Useful in rate-multiplier conversations.

12 **Specific ask, specific fallback, specific walk-away.** What are you asking for? What will you accept if the primary ask is declined? At what point will you decline the offer and pursue alternatives? All three numbers must be clear before you enter the conversation — not after.

BACK MATTER

Pre-Negotiation Worksheet

Fill this out 24 hours before the conversation. If you can't complete every field, you are not ready.

YOUR NUMBERS

Current total cash compensation (base + production + bonuses): $_____

Current 12-month wRVU production: _____ wRVUs

Effective rate per wRVU (total comp ÷ wRVUs): $_____

Specialty category (Hospitalist: IM / Nocturnist / IM: General / Ambulatory): _____

PUBLISHED 2025 BENCHMARK FOR YOUR SPECIALTY

25th: $_____ Median: $_____ 75th: $_____

Your position on the curve: _____

THE ASK

Primary ask (specific number, not a range): $_____

Acceptable counter-offer (your floor): $_____

Walk-away (below this, you decline): $_____

OBJECTION PREP

Most likely employer deflection (from Part II): _____

Your prepared response (memorized verbatim): _____

MEETING LOGISTICS

Meeting date / time: _____ Decision-maker: _____

Follow-up email deadline (within 48 hours): _____

Tear out, photocopy, or print this page. Bring the completed worksheet to the meeting.

BACK MATTER

Methodology & Sources

Methodology. Compensation benchmarks reflect 2025 published surveys (Doximity, Medscape, MGMA). RVU and Medicare reimbursement values reflect CMS CY2026 PFS Final Rule, January 2026 release. Data last reviewed: April 2026.

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